FOR WOMEN 40+ ON GLP-1 MEDICINES

The Muscle Tax

Every pound you lose can come with a bill the scale never shows. Here is what it is, and the 3 habits that keep it low.

The Kira Method · 4-week plan · Your protein number · 25-minute strength routine · Doctor questions

READ THIS FIRST

Who this is for, and who it is not

This guide is for women who take, or are about to start, a GLP-1 medicine (such as semaglutide or tirzepatide) and want to lose fat **without losing more strength than they have to**.

This is education, not medical advice.

It does not diagnose, treat or replace your doctor. Your prescriber decides your medicine and your dose. Never change, skip or stop a medicine because of something you read here. If you have kidney disease, diabetes, heart disease, an eating disorder, or you are pregnant or breastfeeding, talk to your doctor or a registered dietitian before you change how much protein you eat or start a workout plan.

What you will get:

- What the "muscle tax" is, why the scale hides it, and what "lean mass" means.
- A protein target you can calculate in one minute, plus easy ways to hit it when you barely feel hungry.
- A 25-minute strength routine you can do at home, three times a week.
- What to do about nausea, constipation, dehydration and low energy, and the signs that mean "call your doctor now."
- An honest chapter about your face, skin and hair.
- A 4-week plan, a tracker, and the exact questions to take to your next appointment.

The idea in one line: the medicine lowers your appetite. Your job is to make sure that the smaller amount you eat, and the way you move, tell your body to *keep* muscle and lose fat.

CHAPTER 1

The bill nobody shows you

When you lose weight, you lose fat, but you can also lose lean mass. Lean mass is everything in your body that is not fat: muscle, but also water, organs and bone. Muscle is the part you can protect most.

> **The Muscle Tax**
>
> Think of it as a tax on weight loss. When you lose weight, the scale adds up fat and lean mass together. Some of what comes off can be muscle. The tax is not a law of nature and it is not the same for everyone. You can lower it, and this guide shows you how.

Studies that scanned people's bodies during treatment with semaglutide and tirzepatide found that a meaningful share of the weight lost was lean mass. Depending on the study and the way it is counted, reports put it somewhere between about a quarter and about 40 percent of the weight lost. These numbers come from clinical trials, and they are not a promise about *your* body. They show why this topic matters, and they are the reason that medical groups now talk about protecting muscle during treatment.

> **Why this matters more after 40.** Muscle already declines with age. UCLA Health reports that people can lose roughly 3 to 8 percent of muscle mass per decade from their 30s, faster around menopause. Muscle helps you carry groceries, climb stairs, keep your balance and keep your joints steady. Losing it while you are also eating less can leave you thinner but weaker.

What the experts say

In 2025 four U.S. medical organizations (the American College of Lifestyle Medicine, the American Society for Nutrition, the Obesity Medicine Association and The Obesity Society) published a joint advisory on nutrition during GLP-1 treatment. One of its priorities is enough protein together with strength training, so that you keep lean mass. This guide turns that idea into daily steps.

The good news

You have more control than you think. Two habits do most of the work: **enough protein** and **regular strength training**. The rest of this guide shows you how to do both even on days when food sounds terrible.

CHAPTER 2

How to lower the tax: the Rule of Three

Lever	What to do	Target
1. Protein	Eat protein at every meal, even a small one. Protein first, then the rest.	About 20–30 g per meal
2. Strength	Make your muscles work against resistance: bodyweight, bands or dumbbells.	2–3 sessions a week
3. Fuel & fluids	Drink enough, eat small and often, treat side effects early, sleep.	Pee pale yellow, no skipped days

If you only have energy for one thing this week, pick protein. If you have energy for two, add strength. The third lever keeps the first two possible.

A mindset shift. Do not ask, "How little can I eat?" Ask, "How do I make every bite count?" Your medicine already handles the "less." You handle the "better."

CHAPTER 3

Protein: your number, and how to reach it

Step 1. Find your daily range

The basic U.S. recommendation for adults is 0.8 grams of protein per kilogram of body weight per day. That amount is the minimum to avoid deficiency, not a target for muscle protection. For people losing weight on GLP-1 medicines, experts commonly suggest more, often in the range of **1.2 to 1.6 grams per kilogram per day**. In pounds, that is about **0.55 to 0.7 grams per pound**.

Your weight	Daily protein range (approx.)
130 lb (59 kg)	71 – 94 g
150 lb (68 kg)	82 – 109 g
170 lb (77 kg)	93 – 123 g
190 lb (86 kg)	103 – 138 g
210 lb (95 kg)	114 – 152 g

> **Check your number with your doctor or dietitian first,** especially if you have kidney disease, because higher protein is not right for everyone. If you are heavier, your dietitian may base your target on a goal weight instead of your current weight.

Do not panic if the top of the range looks impossible. Start at the bottom of your range and build up over two weeks.

Step 2. Spread it out

Aim for roughly **20 to 30 grams at each meal**, plus a protein snack if your appetite allows. Your body uses protein better in smaller doses through the day than in one big dinner, and when your appetite is low, small servings are easier to finish.

Step 3. Protein cheat sheet (approximate)

Amounts are typical values. Brands differ, so read the label.

Food	Portion	Protein
Chicken breast, cooked	3 oz (palm-sized)	~26 g
Canned tuna	3 oz	~20 g
Salmon, cooked	3 oz	~19–22 g
Eggs	1 large	~6 g
Plain Greek yogurt, nonfat	3/4 cup (6 oz)	~15–17 g
Cottage cheese	1/2 cup	~12–14 g
Milk	1 cup	~8 g
Lentils, cooked	1 cup	~18 g
Firm tofu	1/2 cup	~10 g
Cheddar cheese	1 oz	~7 g
Almonds	1/4 cup	~7 g
Protein powder (whey or soy)	1 scoop	~20–25 g (check label)

Step 4. When you are just not hungry

- **Protein first.** Eat the protein part of the meal first, while you still have room. Vegetables and starch come after.
- **Drink part of it.** A protein shake, kefir or a glass of milk counts. Sip slowly over 20 to 30 minutes.
- **Make it cold and bland.** Greek yogurt, cottage cheese, a hard-boiled egg or cold chicken often feel better than hot, rich food.
- **Set an alarm.** Eat by the clock, not by hunger, three small meals and one or two snacks.
- **Pre-pack.** Hard-boiled eggs, yogurt cups and cheese sticks in the front of the fridge beat a big cooking project at 6 p.m.

A sample low-appetite day (about 85–95 g protein)

Time	What	Protein
Morning	3/4 cup Greek yogurt + 1/4 cup almonds	~22 g
Late morning	1 hard-boiled egg + 1 oz cheese	~13 g
Lunch	3 oz tuna or chicken on greens + a slice of toast	~25 g
Afternoon	Glass of milk or a protein shake	~8–25 g
Dinner	3 oz fish or chicken + vegetables	~22–26 g

CHAPTER 4

Strength: 25 minutes, three times a week

Muscle only stays when it has a reason to stay. Resistance training is that reason. The U.S. physical activity guidelines recommend muscle-strengthening activity on at least two days a week for all major muscle groups, plus 150 minutes of moderate activity such as brisk walking. The advisory above also pairs protein with strength training. Experts often suggest two to three sessions a week during GLP-1 treatment.

> **Before you start,** ask your doctor if exercise is right for you, especially if you have heart disease, joint problems, osteoporosis, dizziness or blood pressure problems. Stop and call your doctor if you feel chest pain, sharp pain, or faint.

Your routine (do 2 rounds, rest 60–90 seconds between moves)

Move	How	Reps
Sit-to-stand	Stand up from a chair and sit back down slowly, arms crossed. Add a bag of groceries held to your chest when it feels easy.	8–12
Wall or counter push-up	Hands on the wall or kitchen counter, body in one straight line, bend elbows. Lower the surface over time.	8–12
Glute bridge	Lie on your back, knees bent, lift hips until your body is in a line from shoulders to knees. Squeeze at the top.	10–15
Row	With a band, or a bag with a few water bottles, pull elbows back and squeeze your shoulder blades.	8–12
Step-up	Step onto a low, sturdy step and stand tall. Hold a rail if you need balance.	8 per leg
Suitcase carry	Walk 30 seconds holding a weight (a full bag or a dumbbell) in one hand, then switch. Stand tall.	2 x 30 sec

How hard?

Pick a weight or version where the last two reps feel hard but your form is still good. That is "two reps left in the tank." Next week, add one or two reps. When you reach the top of the rep range with good form, make the move a little harder.

> **Low-energy rule:** some days you will feel flat. Do the "minimum dose": 10 minutes, one round, no weights. A short session still counts and keeps the habit alive.

Your week

Day	Plan
Mon	Strength routine + short walk
Tue	Easy walk
Wed	Strength routine
Thu	Easy walk
Fri	Strength routine + short walk
Sat–Sun	Move you enjoy: walk, garden, dance. Rest is part of the plan.

A short, easy walk after meals can help your blood sugar. How long it must be has not been proven, so start with whatever feels comfortable.

CHAPTER 5

Fluids, fuel and the rough days

Most GLP-1 side effects are mild and fade as your body adjusts, and they are most common when you start or raise the dose. They still affect how much protein you eat and whether you feel like moving. Treat them early.

Problem	What often helps
Nausea	Eat smaller meals, eat slowly, stop when you feel full, choose cold or plain foods, avoid greasy and very sweet food. Do not lie down right after eating.
Constipation	More water, more fiber from fruit, vegetables, beans and whole grains, and daily movement. Ask your doctor or pharmacist before you use a laxative or stool softener.
Reflux	Smaller meals, no food within a few hours of bed, ask your doctor about options.
Feeling weak or tired	Often from eating too little, dehydration or poor sleep. Check protein, fluids and sleep first, and tell your doctor.

Fluids

Aim to sip water through the day. A simple check: pale yellow urine is a good sign. Dark urine, very little urine, dizziness when you stand up, or a dry mouth can mean you need more fluid. If you are vomiting or have diarrhea and cannot keep fluids down, call your care team the same day, because dehydration can affect your kidneys.

Call your doctor if:

- You have not had a bowel movement in more than 3 days.
- Nausea or vomiting lasts more than a day or stops you from drinking.
- You have pain under the right ribs, pain after fatty meals, or yellowing of your skin or eyes (possible gallbladder problem).
- You pass much less urine than usual, or you have swelling in your legs or ankles with fatigue.

Get emergency care right away for:

- Severe belly pain, especially pain that spreads to your back.
- Repeated vomiting you cannot stop.
- Swelling of the face, lips or throat, hives, or trouble breathing.

This list is general. Follow the guide that came with your own medicine and your prescriber's instructions.

CHAPTER 6

Your face, skin and hair: an honest chapter

Many women say their fear is not the number on the scale but the mirror. Here is what can be said honestly.

- **Face.** Losing weight quickly can reduce fullness in the face, whatever method you use. No food or supplement is proven to reverse it. Slower loss, enough protein, good sleep and sun protection support your skin. If it bothers you, a board-certified dermatologist can talk through options.
- **Skin.** Skin needs protein, fluids, and sun protection (a daily SPF 30 or higher). Loose skin depends on how much weight you lose, how fast, your age and genetics.
- **Hair.** Hair shedding can happen a few months after rapid weight loss or big changes in eating, and it is often temporary. If shedding is heavy or lasts, ask your doctor to check things like iron and thyroid, and review your protein intake.

> No guide can promise you a particular look. What you can do is give your body the building blocks and the movement it needs, and ask for help early.

CHAPTER 7

Your 4-week plan

Week	Focus	Checklist
Week 1	Find your protein number. Put protein at every meal. Start sipping water on a schedule. Do two 10-minute "minimum dose" sessions.	Weight in lb: ____ Protein goal: ____ g
Week 2	Reach the lower end of your protein range most days. Do the full routine twice. Add one easy walk.	Days at protein goal: __ / 7
Week 3	Three strength sessions. Add reps or weight to one move. Track side effects and tell your doctor about any that bother you.	Sessions: __ / 3
Week 4	Keep what works. Take your tracker and questions to your next appointment. Choose what to keep for the next month.	What I will keep: _____

Daily tracker (copy this page, one per week)

Day	Protein (g)	Water (glasses)	Strength?	Energy 1–5	Side effects
Mon					
Tue					
Wed					
Thu					
Fri					
Sat					
Sun					

How to know it is working: not just the scale. Look at: can you do one more rep, is the weight you carry feeling lighter, do clothes fit differently, is your energy steadier. Ask your doctor whether a body composition measurement makes sense for you.

CHAPTER 8

Take this to your next appointment

Your care team wants you to succeed. These questions make the visit productive. Tick the ones you want to ask.

☐ What protein target is right for me, given my weight, kidneys and other conditions?

☐ Should I see a registered dietitian who works with people on GLP-1 medicines?

☐ Is my exercise plan safe for me? Is there anything I should avoid?

☐ Which side effects are expected for my medicine and dose, and which should make me call you?

☐ Should I be checked for vitamin and mineral levels (for example iron, vitamin D, B12) while I lose weight?

☐ I am worried about muscle loss. Does a body composition scan make sense for me?

☐ If my hair is shedding or my energy is low, what should we check?

☐ What is the plan if my weight loss slows down or I need to stop the medicine?

Notes for my visit

Shopping list for week 1. Greek yogurt, eggs, cottage cheese, canned tuna or salmon, a rotisserie or pre-cooked chicken, milk or kefir, firm tofu or lentils, almonds, cheese sticks, frozen vegetables, fruit you tolerate, protein powder (optional), water you enjoy drinking.

SOURCES

Where this comes from

Written October 2026. Research and guidelines change. Check with your own care team. The numbers marked "approximate" are typical values.

- Nutritional priorities to support GLP-1 therapy for obesity: a joint advisory from the American College of Lifestyle Medicine, the American Society for Nutrition, the Obesity Medicine Association and The Obesity Society (2025). Published in the American Journal of Clinical Nutrition and other journals; summary at obesity.org.
- STEP 1 and SURMOUNT-1 body composition (DXA) substudies of semaglutide and tirzepatide: reported lean mass share of weight lost roughly a quarter to 40 percent, varying by study and method.
- CDC, Physical Activity Guidelines for Adults: 150 minutes moderate activity per week and muscle-strengthening activity on at least 2 days per week.
- UCLA Health, "How muscle mass changes after menopause and what you can do about it" (muscle loss of roughly 3–8 percent per decade).
- American College of Lifestyle Medicine, patient handout "Understanding Side Effects of GLP-1 Medicines" (2026).
- KFF Health Tracking Poll (November 2025): about 12 percent of U.S. adults currently take a GLP-1 medicine.

> This guide is educational. It is not medical advice, diagnosis or treatment, and it does not create a doctor–patient relationship. Talk to your doctor before you change medicine, supplements, diet or exercise. Individual results vary and no specific outcome is promised.

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